

*** SYNTHETIC TEST DOCUMENT — NOT A REAL RFP *** *** Fabricated for pipeline testing.
Harmonized to Protocol NEOD001-CL002. *** *** Do not mix into any real document set. ***

June 2, 2015

RE: Request for Proposal ("RFP") — Clinical Trial Study Management for "A Phase 3 Study of NEOD001 Plus Standard of Care for the Treatment of Subjects with Light Chain (AL) Amyloidosis"

Dear Sir or Madam:

Prothena Therapeutics Limited would like to invite your organization to submit a proposal and cost estimate for the conduct of a Phase 3 clinical study in light chain (AL) amyloidosis. As you know, Prothena is a biotechnology company focusing on the discovery and development of novel therapies for amyloid and cell-adhesion related diseases.

This study will evaluate the efficacy and safety of NEOD001 plus standard of care versus placebo plus standard of care in subjects with AL amyloidosis. NEOD001 is an investigational monoclonal antibody that targets misfolded light chain protein. Prothena is developing NEOD001 as an agent for the treatment of subjects with AL amyloidosis.

We are seeking a full-service contract research organization (CRO) that has the capability to manage and perform site coordination, site initiation, on-site monitoring, randomization, site contracting and payment management, biometrics, data management and reporting services, and management of central and core laboratory services.

This RFP consists of an introduction, the proposal instructions, a study synopsis, study overview, work breakdown, timeline, and contractual terms and conditions. All instructions and requirements should be followed to ensure that a proper evaluation can be performed.

This RFP contains information which is confidential to Prothena and must be handled in accordance with the terms and conditions in the Confidentiality Agreement.

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The anticipated schedule for this proposal and selection process is as follows:

June 16, 2015 RFP Response due back to Prothena

July 1, 2015 RFP Award

July 31, 2015 Finalized Contract and Statement of Work

Your completed proposal should be delivered in person or by express delivery to the undersigned no later than 5:00 PM, June 16, 2015. Proposals received after this time may not be considered.

Sincerely,

Clinical Research Associate Prothena Therapeutics Limited

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Attachment II: Request for Proposal

STUDY SYNOPSIS:

Study Title: A Phase 3, Randomized, Multicenter, Double-Blind, Placebo-Controlled, 2-Arm, Efficacy and Safety Study of NEOD001 Plus Standard of Care vs. Placebo Plus Standard of Care in Subjects with Light Chain (AL) Amyloidosis

Protocol Number: NEOD001-CL002

Regulatory IDs: NCT02312206; EudraCT 2014-003865-11

Study Objectives: Determine whether treatment with NEOD001 plus standard of care, compared with placebo plus standard of care, improves time to all-cause mortality or cardiac hospitalization in subjects with AL amyloidosis.

Evaluate the effect of treatment on cardiac and renal organ

response, functional status, and quality of life.

Characterize the safety and tolerability of NEOD001 in this

population.

Study Design: Randomized, double-blind, placebo-controlled, multicenter, 2-arm parallel treatment groups. Subjects randomized 1:1.

Approximately 260 subjects (130 per arm) will be randomized

to one of two treatment groups:

Group 1: NEOD001 + standard of care 130 subjects

Group 2: Placebo + standard of care 130 subjects

Test Agent: NEOD001, an investigational monoclonal antibody

Placebo: Matching placebo for NEOD001

Efficacy Assessments: Time to all-cause mortality or cardiac hospitalization, adjudicated by a Clinical Events Committee. Cardiac and renal biomarker response over the study period.

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Attachment III: Request for Proposal

STUDY OVERVIEW:

Institutional Review Board Sites will use their local IRB

Study Duration 40 month study duration; enrollment over
approximately 24 months

Site Qualification Visits & 1 visit per site; a total of 80 visits to

Time on Site qualify 75 sites; an average of 5 hours
per visit

Investigator Study Start-up A 1.5 day meeting, held prior to first
Meeting site initiation

Site Initiation Visits & 1 visit per site for a total of 75 visits;

Time on Site an average of 6 hours per visit

Number of Subjects	260 (130 per arm)
Number of Sites	75
Site Monitoring Frequency	Approximately every 6 weeks or per every 1-2 subjects enrolled
Interim Monitoring Visits &	10 visits per site for a total of 750
Time on Site	visits; an average of 16 hours or 2 days per visit
Close-out Visits & Time	1 visit per site for a total of 75 visits; on Site an average of 12 hours or 1.5 days per visit
Pharmacy Monitoring Frequency	Initial visit after 1-2 subjects enrolled then approximately every 8 weeks
Pharmacy Monitoring Visits &	4 visits per site for a total of 300 visits;
Time on Site	an average of 4 hours per visit
Number of SAEs Anticipated	Approximately 400
Case Report Forms	Approximately 80 pages per subject
Data Management	Approximately 20,800 CRF pages total
Interim Efficacy Analysis	Performed after approximately 156 primary endpoint events
Statistical Analysis	CRO to perform a final statistical analysis of the data to include 45 tables, 38 listings, and 12 figures
Clinical/Statistical Report	CRO to write integrated report

This timeline is based on approval of the protocol by the FDA and IRB and subject accrual.

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Attachment IV: Request for Proposal

TASK AND RESPONSIBILITY LIST (summary):

- 1 — Regulatory (IND updates, FDA communication, site registration, IND safety reports)
- 2 — Protocol Preparation (study design support, protocol writing, informed consent drafting)
- 3 — Case Report Form Preparation (design and print CRFs to sponsor standards)
- 4 — Pre-study Preparation (investigator identification, site evaluation, site selection, IRB submissions, regulatory document collection)
- 5 — Investigator Meeting (planning and conduct)
- 6 — Study Initiation (site regulatory binders, initiation visits and reports)
- 7 — Site Monitoring (on-site monitoring, written reports, weekly site contact, regulatory binder maintenance)
- 8 — Data Management (database build, entry, cleaning, reconciliation)
- 9 — Biometrics (randomization, statistical analysis, report writing)
- 10 — Central Laboratory Coordination

Services requested: site coordination, site initiation, on-site monitoring, randomization, site contracting and payment management, biometrics, data management and reporting services, and management of central and core laboratory services.

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*** END OF SYNTHETIC TEST DOCUMENT ***